

Physician, patient, team and law

The chronically ill patient, models of the relationship, interprofessional relations and the physician's legal liability

Practical Class 4 · Social Medicine and Medical Ethics I · 2026/2027

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Four questions for today's class

1. The chronically ill patient
2. Models of the physician–patient relationship
3. Interprofessional relations
4. The physician's legal liability

The chronically ill patient

Chronic illness changes the logic of care

Dimension	Acute care	Chronic care
Time	A limited episode of treatment	Long-term or lifelong follow-up
Goal	Control the danger, restore health	Control, preserved function, acceptable quality of life
Patient	Receives help, follows a short plan	Makes daily decisions, carries out much of the care
Physician	Diagnoses and acts promptly	Sustains the relationship, agrees goals, coordinates

Disease is the diagnosis; **illness** is the experience of it. Good care addresses both.

How patients cope with illness

Problem-focused

Seeking information, self-monitoring, changing routines, planning, asking for help.

Emotion-focused

Distancing, denial, distraction, sharing feelings, religious support.

Meaning-focused

Reordering goals, new roles, values beyond the illness.

A diagnosis splits life into “before” and “after”. The physician supports adaptation and does not grade the “right” reaction.

Common reactions and the physician's response

Reaction	What it looks like	Professional response
Denial	"It isn't that serious for me."	Information in stages; check for dangerous omissions
Anger	Blaming the team or relatives	Name the emotion; do not take it personally
Regression	Passivity, total dependence on others	Keep skills; small agreed tasks
Hopelessness	"There is no point in treatment."	Assess pain, depression, self-harm risk

Reactions recur with each new complication and are not moral failings.

Autonomy, adherence and treatment burden

Relational autonomy

Dependence on a machine, drugs or relatives does not remove the right to choose. The choice may still be shaped by guilt, fear or pressure.

Treatment burden

Drugs, measurements, visits, travel, costs. How much more can this person carry?

Adherence, not compliance

Non-adherence may be unintentional or intentional. Look for the cause, not for someone to blame.

Shared responsibility

Physician: a sound, clear plan. Patient: honest feedback. System: accessible care.

The family: support, burden and limits

What the family brings

- Daily care and observation.
- Knowledge of the patient's life.
- Emotional and practical support.

What the risks are

- Speaking for the patient.
- Overprotection, loss of skills.
- Exhaustion of the main carer.
- Neglect or abuse.

The patient decides which relatives take part and what they learn.

Case 1 · My doctor is an old friend

A patient with newly diagnosed hypertension has known his doctor since childhood. Two months into treatment his blood pressure is higher. He takes the tablets only when he thinks fit and says, laughing: “We’ve known each other for years — just prescribe me stronger pills.”

- What explains his behaviour: lack of discipline or his model of the illness?
- Which relationship model fits?
- Does the friendship help or hinder?

Models of the physician–patient relationship

Szasz and Hollender: three degrees of participation

Model	Division of roles	Typical situation
Activity – passivity	The physician acts; the patient cannot take part	Resuscitation, coma, general anaesthesia
Guidance – cooperation	The physician leads; the patient knowingly follows	Acute illness with a clear standard of treatment
Mutual participation	Tasks and decisions are shared	Chronic illness, rehabilitation, behaviour change

The model follows the patient's condition and changes with it.

Emanuel and Emanuel: four models

Model	Patient's values	Physician's role	Autonomy as
Paternalistic	Objective, set by the physician	Guardian	Assent to “the best”
Informative	Known and fixed	Technical expert	Choice after information
Interpretive	Unclear, need clarifying	Counsellor	Self-understanding
Deliberative	Open to discussion	Teacher, friend	Moral self-development

Elements of more than one model can appear in a single consultation.

Veatch: four models and other variants

Model	Essence	Main risk
Engineering	The physician is a scientist who hands over the facts	The patient's values stay outside the conversation
Priestly	The physician decides as a moral authority	Paternalism
Collegial	Two equals with a common goal	An illusion of equality and shared interests
Contractual	Negotiated rights and duties	Minimalism: "only what was agreed"

The textbook adds: **market** (health as a commodity), **covenant** (a promise of fidelity), **friendship** (blurred boundaries).

Paternalism and professional limits

Strong paternalism

Restricts a capable, informed patient.
Concealing a diagnosis or overriding a refusal is not justified.

Weak paternalism

Protects a person with impaired capacity for the specific decision; lasts only while that ground exists.

- The patient may refuse offered treatment but cannot demand harmful or needless treatment.
- The physician may decline treatment when trust is absent, but never emergency care (Code, Arts. 11, 12).

Shared decision-making

1

**There is a
choice**

2

**Reasonable
options**

3

**What matters to
the patient**

4

**Reasoned
recommen-
dation**

5

Plan and check

- Health Act, Art. 86(1): the right to clear information in accessible language and to more than one medical opinion.
- Health Act, Art. 190(1): freedom of action and decision according to qualification, medical standards and medical ethics.

A recommendation without taking away the choice.

Interprofessional relations

Collegiality in the Code of Professional Ethics

Article	Content
Art. 40	Collegiality is a basic duty, observed while respecting the interests of the patient.
Art. 41	Moral support; no slander or disparagement; disagreements are not matters for public polemics.
Art. 42	It is unworthy to discredit a colleague by criticising their treatment or their person.
Art. 46	Ethical disputes between physicians are decided by the ethics committees of the Bulgarian Medical Association.
Art. 49	A physician helps a colleague who falls ill and cooperates with those treating them.

Consultation, medical council and continuity

Consultation

Sought when the condition requires it (Art. 35). The consultant reports to the treating physician (Arts. 36, 45).

Medical council

For difficulties in diagnosis or treatment; a unanimous decision is binding (Art. 38).

Continuity

Full information on withdrawal (Art. 24); changes made in the treating physician's absence are documented (Art. 50).

Gaps in handover are a common cause of preventable harm.

Physician, nurse and team

Different roles, one patient

Each profession has its own competence, professional body and code of ethics.

Clear orders

Written, legible, checkable. A doubt is clarified, not acted on silently.

A voice for everyone

Any team member may stop a procedure when safety is at risk.

Respect

Humiliating colleagues in front of patients destroys trust and hides errors.

Collegiality does not mean cover-up

Situation	Risk	Professional conduct
A colleague asks for help	Incomplete examination, no records	Formal consultation and follow-up
A colleague learns the diagnosis	Gossip out of curiosity	The same confidentiality as for any patient
Illness affects the work	Cover-up or automatic exclusion	Individual assessment, proportionate limits
Immediate risk	Delay in the name of collegiality	Protect patients and help the physician

Case 2 · The surgeon with a tremor

An experienced surgeon notices a fine hand tremor and asks a neurologist friend to examine him informally after work. The examination raises suspicion of a progressive disease. The surgeon refuses documentation and further tests. Tomorrow he has a complex planned operation.

- What is the status of the informal examination?
- How do confidentiality, collegiality and patient safety fit together?
- What should the neurologist do?

The physician's legal liability

Four kinds of legal liability

1. Disciplinary

Labour Code: disciplinary penalties; employee financial liability towards the employer.

2. Administrative

Administrative violations and fines; subtype: **professional body liability** (BLS / BZS).

3. Criminal

Criminal Code: personal liability for a crime; the court imposes punishment.

4. Civil

Obligations and Contracts Act: compensation; **contractual** and **tort / non-contractual**.

The first block groups two Labour Code regimes; employee financial liability has its own legal basis (Art. 203(3)).

1. Disciplinary liability

Disciplinary penalties

Basis: Labour Code, Arts. 186–188 — culpable breach of work duties.

Reprimand, warning of dismissal, disciplinary dismissal.

Who: employer or authorised person (Art. 192), after explanations (Art. 193).

Employee financial liability

Basis: Labour Code, Arts. 203 onwards — loss to the employer; negligence is subject to the limits in Art. 206.

Who: employer by order; court if contested (Art. 210). Full financial liability is enforced by a court (Art. 211).

Example: a physician arrives late for a shift; separately proven loss to the hospital may also require compensation.

2. Administrative liability

Violations and fines

Basis: Administrative Violations and Penalties Act, Arts. 6, 13, 47; Health Act, Arts. 221, 229.

Who: the competent authority; for Art. 221, the Medical Supervision executive director (Art. 233a).

Example: an establishment violates a statutory patient right.

Subtype: professional body liability

Basis: Professional Organisations Act, Arts. 37–41; ethical code. Reprimand, fine, temporary removal from the register.

Who: regional board chair of BLS / BZS, after an ethics committee member records the breach (Art. 40).

Example: a physician discredits a colleague before patients.

3. Criminal liability

Basis • Criminal Code

Art. 11: intent or negligence; negligence is punishable where the law provides.

Arts. 123 and 134: death or grievous / moderate bodily injury through ignorance or negligent practice of a dangerous occupation.

Who imposes punishment

The court decides guilt and imposes punishment. The prosecution brings and maintains the charge in public prosecutions.

The conduct, causal link to the harm and fault must be proved.

Example: a negligently administered wrong dose causes death. Art. 123 applies if all elements of the offence are proved.

4. Civil liability

Contractual · OCA, Arts. 79–82

Failure to perform a contractual duty properly; damages under Art. 82.

Example: an agreed investigation is not performed and the patient incurs proven additional costs.

Tort / non-contractual

OCA, Art. 45: culpably caused harm; fault is presumed.

Art. 49: liability of the healthcare establishment as principal.

Example: an incorrectly given injection injures a patient's nerve.

The court awards compensation on the injured party's claim. Contract and tort are two subtypes of civil liability.

Case 3 · A handover without the new symptom

At the end of his shift on 24 December a physician waits almost an hour for a late colleague. A nurse reports chest pain in a patient with pneumonia. The physician orders an ECG, but in the rushed handover forgets to mention the symptom. Later the patient dies of acute myocardial infarction.

- Apply the four kinds of liability: which may arise, and for whom?
- What must be proved for civil and for criminal liability?
- What share belongs to the organisation?

Trust is built in the relationship, the team and the institution.

The law sets out the consequences when it is broken.